



AI Playbook and Toolkit for Public Health Departments

AI Workflow Readiness and Operational Planning

PGM 110

This module helps program and operations staff determine whether a workflow is ready for AI-supported change. It focuses on workflow mapping, readiness assessment, role clarity, data dependencies, approved-tool awareness, human review, and operational planning. The module supports teams that need to move from an AI idea to a practical implementation plan without assuming that AI is always the right solution.

Level	applied
Primary track	Program Management Track
Appears in tracks	program-management

Learning Objectives

- Assess whether a public health workflow is ready for AI-supported change.
- Map tasks, handoffs, decisions, data inputs, human review points, and failure points in an existing workflow.
- Identify readiness gaps that should be addressed before AI implementation.
- Develop an operational readiness plan for an AI-supported workflow.

Module Overview

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Jurisdiction and Agency Policy Note

This national-level training module is intended for public health education, planning, and workforce development. It does not replace agency policy, legal review, privacy review, security review, procurement review, communications clearance, civil rights review, Tribal consultation, or governance approval. Learners should confirm applicable requirements, approved tools, data rules, and review pathways within their own jurisdiction and organization before implementing AI-supported workflows.

Why This Topic Matters for Public Health AI

AI systems are implemented into workflows, not into abstract strategies. A workflow may seem ready because staff are excited or a tool is available, but operational readiness depends on clear tasks, reliable data, defined roles, documented handoffs, user training, and review pathways. If the workflow is already unclear, inconsistent, or poorly documented, AI may amplify confusion rather than improve performance. Readiness assessment helps teams decide whether to proceed, redesign, or pause.

Operational planning is especially important in public health because workflows often involve multiple programs, partners, data systems, legal authorities, and public-facing consequences. AI may change who reviews information, how quickly decisions are made, what gets documented, and how errors are detected. Staff need to understand what will change before a tool is deployed. A readiness plan makes those changes visible and gives governance and leadership a basis for decision-making.

Public Health Example

An immunization program wants to use AI to summarize provider questions from a shared inbox and route them to the right team. The operations lead maps the current workflow and discovers that categories are inconsistent, escalation criteria are informal, and some messages include sensitive provider or patient information. Before selecting a tool, the team defines categories, creates a no-sensitive-data handling rule, identifies reviewers, and tests the workflow with fictional examples. The readiness process shows that process redesign must occur before AI can be safely piloted.

Technical, Programmatic, and Operational Deep Dive

Workflow readiness begins with mapping the current state. Teams should document the trigger, inputs, tasks, handoffs, decision points, data systems, approvals, outputs, and pain points. They should identify where delays, duplication, errors, or inequities occur and whether AI would address those issues. The map should include informal workarounds because those often reveal risks that formal procedures miss.

The future-state workflow should identify exactly where AI would enter the process. The plan should explain what the AI would receive, what it would produce, who would review the output, what the human reviewer would check, and what actions are prohibited without approval. It should also specify which tool or environment is approved for the task and which data are not allowed. If approved tools vary by agency, the plan should include a local confirmation step rather than naming a specific tool.

Operational readiness also includes people and support. Staff need training, supervisors need expectations, and users need a way to report errors or concerns. The team should define success metrics, monitoring cadence, issue escalation, and fallback procedures if the AI tool is unavailable or produces questionable outputs. These details should be documented before the workflow moves to pilot or implementation.

Risks, Failure Modes, and Guardrails

A common failure mode is implementing AI into a poorly understood workflow. If tasks, roles, and handoffs are unclear, AI may create faster confusion rather than better performance. Guardrails include current-state mapping, future-state design, user review, and readiness gap tracking. The team should resolve critical gaps before implementation begins.

Another risk is failing to define human review. Staff may assume that someone else is checking outputs, or they may overtrust AI because it appears confident. Guardrails include explicit review points, review checklists, override rules, and documentation requirements. Supervisors should confirm that reviewers have enough time and skill to perform the review.

A third risk is ignoring fallback and incident procedures. If the AI tool fails, produces harmful output, or becomes unavailable, the program still needs to operate. The readiness plan should include fallback workflows, issue reporting, escalation, and pause criteria. This ensures that AI does not become a single point of failure for public health operations.

Application to AI-Supported Workflows

This module applies when a team is considering AI for case routing, document review, inbox triage, dashboard summarization, referral follow-up, inspection prioritization, data quality review, or other operational workflows. The team should map the workflow before selecting or configuring a tool. The readiness plan should be reviewed by governance, IT, privacy, security, equity, and program leadership as appropriate. The result should be a practical decision about whether to proceed, redesign, defer, or stop.

Reflection Questions

Which workflow in your program would need better documentation before AI could be considered?

Where would human review be required in an AI-supported workflow?

What fallback process would be needed if the AI tool failed or produced questionable output?

Practical Exercise

Create an AI workflow readiness plan for one public health workflow. Include a current-state workflow summary, proposed AI support, data inputs, approved-tool confirmation step, human review points, readiness gaps, training needs, monitoring measures, fallback process, and next decision point.

Example: For AI-supported inbox triage, the plan might identify incoming messages, classification categories, prohibited sensitive data, reviewer responsibilities, routing rules, error reporting, and fallback to manual triage if the tool misclassifies urgent messages.

Expected Artifact or Evidence

Workflow readiness plan.

Current-state and future-state workflow summary.

List of readiness gaps and owners.

Knowledge Check

- 1. Why should workflows be mapped before AI implementation?
- 2. What is a readiness gap?
- 3. What should a human review point specify?
- 4. Why is a fallback process important?
- 5. When should approved-tool awareness be addressed?

References and Resources

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